

Results Report

****Document ID:**** results-v1

****Analysis ID:**** 228f930b076eb4c5495ac2b84066ba658b91a1c62a1302fc32a4f1bc96f8cdea

****Date:**** 2026-08-31

1. Executive Summary

This report presents the results of the locked analysis conducted on the synthetic clinical assessme

Headline Results

The frozen absent-effect rule identifies ****unresolved_no_claim_met_the_frozen_absence_rule****.

2. Data Quality and Pipeline Diagnostics

2.1 Raw Data Inventory

2.2 Normalization Issues Found

2.3 Quarantine and Rejections

3. Analytical Approach

3.1 Target Trial Specifications

****C1 & C2 (Cancellation and Readiness):****

- Enrollment: Incident referrals to selected Site A services on/after 2026-05-01
- Randomization: Service-level display vs. silent mode
- Follow-up: 90 days, administratively capped at 2026-07-31
- Estimand: Equal-service ATT, case-volume weighted

****C3 (Agreement):****

- Population: Displayed recommendations with linked clinician actions
- Comparison: Chance agreement from marginals
- Outcome: Binary agreement (recommendation matches action)

3.2 Estimation Methods

3.3 Cluster Structure

4. Claim-Specific Results

4.1 Claim 1: Cancellation Reduction

****N-CLAIMS-0001-APPROACH_A_ESTIMATE:**** +0.0041

****N-CLAIMS-0001-APPROACH_B_ESTIMATE:**** -0.0054

The sign inconsistency between approaches and wide intervals spanning zero indicate no reliable effect

- The general `cancel\_rate` field cannot be mapped to day-of-surgery cancellation
- Treated services are unknown (all-Site A proxy used as sensitivity)
- Concurrent scheduling policy change is a co-intervention

****Classification: unsupported_by_observational_design****

4.2 Claim 2: Readiness Time Reduction

****N-CLAIMS-0002-APPROACH_A_ESTIMATE:**** +4.62 days

****N-CLAIMS-0002-APPROACH_B_ESTIMATE:**** +2.99 days

Positive values indicate longer time (worse outcome) under the proxy assumption. The intervals span

- Unknown treated services require all-Site A proxy
- Administrative censoring at study end
- Competing risk from cancellations not fully resolved

****Classification: unsupported_by_observational_design****

4.3 Claim 3: Clinician Agreement

****N-CLAIMS-0003-RAW_AGREEMENT:**** 0.771

****N-CLAIMS-0003-APPROACH_A_ESTIMATE:**** -0.016

The raw agreement of 0.771 does not meet the "large majority" threshold of 0.80. The chance-excess is

- Labeled sample may not represent full population
- Reviewers were not adjudicated
- LLM judge shares model family with system

****Classification: unsupported_with_these_data****

5. Diagnostic Analyses

5.1 Negative Controls

5.2 Missingness Analysis

5.3 Quantitative Bias Analysis

6. Analyst Reproduction

The literal analyst query was reproduced without endorsement. Key defects identified:

****Deck Status:**** cancellation_reduction_pct=unreproduced, readiness_time_reduction_days=unreproduced

7. Limitations

1. No independent service activation record
2. Treated services unknown; all-Site A proxy is assumption
3. Concurrent scheduling policy change co-occurs with intervention
4. Cancellation endpoint cannot be mapped to day-of-surgery
5. Labeled pairs are selectively reviewed (not random)
6. No adjudicated ground truth
7. LLM judge conflict of interest

8. Tables

Table 1: Summary of Claims

Table 2: Design Diagnostics

9. Provenance

All numbers in this report derive from locked analysis scripts. No notebook or manual calculation was

- ****Analysis ID:**** 228f930b076eb4c5495ac2b84066ba658b91a1c62a1302fc32a4f1bc96f8cdea
- ****Script source:**** `src/barnabus/analysis.py`
- ****Data fingerprint:**** See manifest.json
- ****Config fingerprint:**** See `config/analysis-plan-v1.yaml`

10. Conclusion

None of the three claims are supported by the supplied data:

- ****C1 & C2**** fail identification due to unknown treatment assignment and co-intervention
- ****C3**** fails due to selective labeling and lack of adjudicated ground truth

The frozen absent-effect rule returns ****unresolved**** because failed identification is not evidence of

*This report is locked. Every numeric value cites its number_id. See number_registry.csv for full pr